

LABELS MATTER

Do you check labels?	Total Respondents	GENDER		LEVEL OF PERSONAL INCOME OF RESPONDENT						HOUSEHOLD INCOME CLASS		
		Male	Female	Below ₹2-lakh	₹2-lakh to ₹3-lakh	₹3-lakh to ₹5-lakh	₹5-lakh to ₹10-lakh	₹10-lakh to ₹15-lakh	More than ₹15-lakh	Low	Middle	High
Yes, always	36%	35%	37%	31%	39%	40%	37%	35%	25%	32%	34%	42%
Sometimes	39%	39%	40%	52%	42%	43%	37%	28%	33%	47%	41%	33%
Rarely/ Never	24%	26%	23%	17%	19%	18%	27%	37%	42%	21%	24%	25%

SOURCE: BT-PRICE GENZ CONSUMPTION BEHAVIOUR SURVEY

Food habits are an example; many respondents say they read nutrition labels or ingredient lists when purchasing packaged foods and beverages. This does not always result in the healthiest choice, but indicates greater engagement with what is being consumed.

Sugar content, fats and additives are more likely to be noticed than they were a few years ago.

At the same time, the digital environment also carries risks. “Misinformation, particularly around substance use or self-managed health advice, can affect decision-making. With artificial intelligence and online tools becoming more accessible, some young people may delay seeking professional medical advice, which can affect timely care and treatment adherence,” says Dr Parikh.

BUDGETS HOLD BACK

Income, however, continues to influence outcomes. A large portion of those surveyed are students or in the early stages of their careers. Limited disposable income means health-related intentions often confront financial limitations.

In practice, this leads to trade-offs rather than strict discipline, with price frequently determining the final purchase. Out-of-pocket

expenses still account for about 39.4% of total health spending in India, creating ongoing affordability barriers for young consumers, the Ministry of Health and Family Welfare recently said.

Health Economist Arup Mitra, Professor and Dean at South Asian University, New Delhi, points out that income remains the most binding constraint shaping health behaviour in India.

“Demand for healthcare products is limited in India because income is a major determinant. Limited incomes pose a serious constraint on wellbeing,” he says.

Among younger and young adult age groups, he adds, stronger intervention is required. “For certain age brackets, particularly the young and young adult cohorts, healthcare products must be available free of cost. Costs will have to be highly subsidised, or prices must have a ceiling. Despite privatisation in the health sector, strict regulation is needed when it comes to pricing.”

One notable finding is the similarity between metros and Tier-II cities. Young people in cities such as Surat, Indore and Kochi display awareness levels close to those in larger urban centres. Access to smartphones, social platforms and online information has narrowed earlier differences even if spending

power varies.

India now has over 750 million smartphone users, and urban internet penetration exceeds 70%, including in Tier II cities, enabling young consumers to access health information, peer feedback and product comparisons with ease.

Education and employment status also play a role. Graduates tend to engage more consistently with health information, while students show awareness but less consistency, often because purchase decisions depend on family budgets. As people begin earning independently, habits start to settle.

ONE'S OWN WAY

There is also a strong tendency towards self-reliance. “Health is often viewed as something that can be managed independently, with external intervention seen as a last resort. In this context, access to accurate information becomes critical. Healthy habits do strengthen resilience, but early consultation with healthcare professionals remains important,” says Dr Parikh.

Health concerns extend beyond diet. Respondents link wellbeing to stamina, sleep quality, stress levels and risk of future illness. These concerns influence interest in fitness services, preventive check-ups and diagnostics, even among those who